

SECTION 2

The 19-Year Mortality Gap

Alberta's widest documented life expectancy disparity, and what it means for a program designed around ageing into benefits

This section establishes a single finding and traces its consequences. The finding is that in 2023, the average life expectancy at birth for First Nations people in Alberta was 62.81 years. In the same year, for non-First Nations people in Alberta, it was 81.88 years. The difference is 19.07 years. It is the widest documented life expectancy gap between First Nations and non-First Nations Albertans on record.

Every subsequent section of this report, and every aspect of the July 1, 2026 ADAP transition, has to be understood against that finding. A provincial disability program being redesigned around the premise that recipients will age into seniors' benefits at 65 is a program whose structural logic does not apply to the First Nations recipients whose average lifespan ends before that point.

Alberta First Nations recipients do not, on average, age out of AISH. They die before it."

The figures

Current state, 2023

The following data points are drawn from the Government of Alberta's own 2023 figures and from peer-reviewed research published in The Lancet Regional Health – Americas in March 2025 by researchers working with First Nations health managers in Alberta:

- First Nations life expectancy at birth, Alberta, 2023: 62.81 years
- Non-First Nations life expectancy at birth, Alberta, 2023: 81.88 years
- Gap: 19.07 years
- First Nations men, Alberta, 2021: approximately 60 years
- First Nations women, Alberta, 2021: approximately 66 years
- Non-First Nations men, Alberta, 2021: approximately 79 years
- Non-First Nations women, Alberta, 2021: approximately 84 years

For context, a life expectancy of 60 years for Alberta First Nations men in 2021 is comparable to the national life expectancy of men in Haiti and Bolivia. The Alberta Indigenous Primary Health

Care Advisory Panel, in its 2022 report, characterized the Alberta First Nations life expectancy as “in alignment with countries like Niger, Angola and Afghanistan and similar to life expectancy witnessed in Canada during the Great Depression in the 1930s.” That comparison is drawn from a Government of Alberta-affiliated advisory body, not from advocacy research.

The trajectory: the gap is widening, not closing

The 19-year gap is not a static disparity. It is a disparity that has been actively growing over the past quarter century, during precisely the period in which federal and provincial governments have publicly committed to closing it.

- 1974–1978: The documented gap between Indigenous life expectancy in Alberta and the total provincial population was 15 years. (Health and Welfare Canada, 1981 study.)
- 1999: The gap had narrowed to approximately 7 years. Life expectancy for First Nations people in Alberta ranged from 69.9 to 72.6 years during the 1999–2019 period, against non-First Nations life expectancy of 79.6 to 82.8 years.
- 2019: First Nations life expectancy in Alberta was 70.4 years. Non-First Nations life expectancy was 82.8 years. Gap: 12.4 years.
- 2021: First Nations life expectancy dropped to 63.1 years. Non-First Nations life expectancy was 81.4 years. Gap: 18.3 years.
- 2022: First Nations life expectancy recovered slightly to 64.2 years. Gap: 17 years.
- 2023: First Nations life expectancy 62.81 years. Non-First Nations 81.88 years. Gap: 19.07 years — the widest on record.

Read in sequence: the gap was narrowing through the 1980s and 1990s, stabilized in the early 2000s, began to widen in the mid-2010s, collapsed precipitously during the COVID-19 pandemic, and has continued to widen since. The peer-reviewed analysis in *The Lancet Regional Health – Americas* attributes this trajectory to a combination of pre-existing systemic inequities — delayed access to care, housing instability, contaminated water, chronic disease prevalence — exacerbated by two acute crises: COVID-19 and the toxic drug poisoning epidemic.

COVID-19: the asymmetric shock

Between 2019 and 2021, life expectancy for First Nations people in Alberta fell by 7.3 years. In the same two-year period, life expectancy for non-First Nations Albertans fell by 1.4 years. The First Nations population experienced more than five times the life expectancy loss of the non-First Nations population during the pandemic.

The peer-reviewed *Lancet* analysis documented that during the pandemic, the mortality rate for young adult First Nations women in Alberta was approximately eleven times higher than for non-

First Nations women of the same age. The 20–50 age groups experienced the highest relative mortality rate ratios. These are not elderly deaths. They are working-age deaths, in a population that was already dying earlier than the provincial average before the pandemic began.

In January 2021, on-reserve COVID-19 case rates were 40 percent higher than the overall provincial population. The reasons identified in the peer-reviewed literature include: overcrowded housing (addressed in Section 4), delayed access to health care (addressed in Section 7), pre-existing chronic disease from environmental and social determinants (addressed in Sections 5 and 6), and contaminated water supplies in some communities (addressed in Section 6). Each of these vulnerabilities was documented before 2020. The pandemic did not create them. It exposed them.

The toxic drug crisis: a 366 percent increase

Between 2016 and 2022, unintentional opioid poisoning deaths among First Nations Albertans increased by 366 percent. In 2016, 188 First Nations Albertans died from unintentional injury of all kinds (including falls, traffic accidents, and poisonings). In 2022, 344 First Nations Albertans died from unintentional opioid poisoning alone.

The rate disparity, in the 2024 Alberta Health surveillance data, is stark:

- 2016–2019: First Nations opioid death rate ranged from 44 to 70 per 100,000 person-years. Non-First Nations: 11 to 16 per 100,000 person-years. A ratio of roughly four-to-one.
- 2020: First Nations 148 per 100,000 vs non-First Nations 20 per 100,000. Ratio: 7.4 to 1.
- 2021: First Nations 224 per 100,000 vs non-First Nations 26 per 100,000. Ratio: 8.6 to 1.
- 2022: First Nations 205 per 100,000 vs non-First Nations 24 per 100,000. Ratio: 8.5 to 1.

The disparity is not attributable to different patterns of substance use. It is attributable primarily to the toxicity of the unregulated drug supply, combined with the absence of harm reduction infrastructure in the communities where First Nations Albertans live, and the closure of specific treatment and harm reduction centres in Alberta under successive provincial governments.

The age distribution of these deaths matters. The largest proportion are among people aged 25 to 44 — the same age band that carries the heaviest work and caregiving responsibilities in First Nations communities, the same age band in which children are being raised, and the same age band that, in the AISH program, represents the recipients most likely to still be in the early decades of an anticipated benefit relationship.

What this means for AISH, and for ADAP

The structural premise of AISH

The Assured Income for the Severely Handicapped program, established in 1979, was designed around a specific demographic assumption. Recipients enter the program between age 18 and age 65, after being medically certified as having a severe and permanent disability. Benefits continue until the recipient either (a) becomes eligible for Old Age Security at age 65, at which point they transition to federal senior benefits supplemented by the Alberta Seniors Benefit, or (b) dies.

The federal Old Age Security pension, the Guaranteed Income Supplement, and the Alberta Seniors Benefit together form what is, for most AISH recipients, a higher monthly income than AISH itself. For a single AISH recipient at the current \$1,940 monthly benefit, the combined federal and provincial senior benefits can total approximately \$2,130 per month. The AISH-to-senior-benefits transition, despite the administrative burden it creates, is generally a financial improvement.

The demographic assumption built into AISH is therefore: most recipients will age into the seniors' benefit system and receive more income as seniors than they received as AISH recipients.

The assumption does not apply to Alberta First Nations

In 2023, the average life expectancy at birth for First Nations people in Alberta was 62.81 years. The AISH program transitions recipients out at age 65. The mathematics are straightforward: on average, Alberta First Nations AISH recipients die 2.19 years before they would have aged out of the program.

Put another way: the federal and provincial senior benefits that compensate for lower AISH payments over a recipient's lifetime, and that make the AISH-to-senior-benefits transition financially acceptable, do not apply to the First Nations recipient population on average. The compensating mechanism is never triggered. The lifetime cost of being on AISH, for First Nations Albertans, is borne entirely within the AISH payment structure itself — because there is no average post-65 phase of life during which the senior benefit structure corrects for it.

The 19-year gap is not only a public health statistic. It is a structural fact about how a disability benefit program operates, and about which populations that program serves adequately versus inadequately. A program built around the assumption of a 65-year-plus lifespan does not serve a population with an average lifespan of 62.8 years. It serves that population on paper. In practice, it under-serves them by design.

The ADAP transition makes this worse

The July 1, 2026 transition from AISH to the Alberta Disability Assistance Program (ADAP) introduces two additional dimensions to this problem.

First, the monthly benefit level under ADAP is \$1,740 — a \$200 reduction from the current AISH rate of \$1,940. A transition benefit maintains the current rate for existing AISH recipients until

December 31, 2027, after which the full \$200 reduction takes effect. For new applicants on or after July 1, 2026, the \$1,740 rate applies immediately. For a First Nations recipient with an average life expectancy of 62.81 years, that \$200 per month over the remaining years of AISH eligibility represents a material reduction in lifetime benefits received — not offset by any corresponding senior benefit, because most of those recipients will not reach the senior benefit window.

Second, ADAP is structured around a premise of employment readiness. Recipients who transition to ADAP, or who are newly enrolled under ADAP, are expected to engage with employment services designed to return them to some form of paid work. For a population with an average life expectancy below the program's own age-out threshold — and with documented rates of chronic disease, environmental exposure, traumatic injury, and mental health burden rooted in the historical context of Section 1 and the contamination reality of Section 5 — the premise of progressive return to employment is not a medical prediction. It is an administrative convenience.

Put directly: ADAP asks First Nations Albertans, whose lifespans place them statistically below the age at which the program was designed to end, to demonstrate their capacity to earn income in a labour market from which they have been systemically excluded for centuries, while living in an environment that has been actively contaminated by industrial and governmental choice. The ask is not realistic. The ask is also not the point. The point of the ask is the administrative filter it creates — the population that cannot meet the demonstration requirement is the population that, under ADAP, sees its benefit reduced and its appeal rights stripped.

What this establishes for the remainder of the report

The remaining sections of this report — jurisdictional fracture, housing, water, dual healthcare systems, Jordan's Principle, child welfare, justice system, missing and murdered women, consultation failure, Sovereignty Act conflict, TRC Calls to Action, and the July 1 transition itself — all operate within the demographic reality established here.

Each subsequent section is a specific documentation of one of the structural drivers of the 19-year gap. Each of those drivers has, in turn, been documented for decades. Each has been the subject of federal commissions, provincial reports, peer-reviewed research, and Indigenous-led advocacy. None has been resolved. The gap has widened during the period of documentation.

This is the structural reality into which the ADAP transition is being introduced, on July 1, 2026, without Indigenous consultation meeting UNDRIP standards, without structural accommodation for the demographic profile of the affected population, and without acknowledgement that the program's founding assumptions do not apply to a significant subset of its recipients.

A program designed around ageing into benefits does not, in operational reality, serve a population that dies before the ageing-in date."

Section 3 addresses the jurisdictional fracture — the federal-provincial-status-non-status split that determines which institutional system is responsible for which Indigenous person at which moment, and how that fracture itself produces the access delays and coverage gaps that contribute to the mortality profile documented here.

Sources cited in Section 2

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